

22STCV31694 22STCV31694

County: los-angeles

Division: Civil Law and Motion

Department: C

Hearing date: 2026-06-30

Source URL: https://www.lacourt.ca.gov/tentativeRulingNet/ui/main.aspx?casetype=civil&dept_date=SE%20%2CC%2C06%2F30%2F2026

Source SHA-256: 83aed9c090f6d202ad3be40d5ca5a694ba7e8a237aba090acfd84e84e673b132

Case Number: 22STCV31694 Hearing Date: June 30, 2026 Dept: C

SOKOL vs NORWALK COMMUNITY HOSPITAL, et al.

CASE NO.:

22STCV31694

HEARING: 6/30/26

@ 9:30 AM

#15

TENTATIVE

RULING

I.

Defendant Kindred Healthcare Operating, LLC's Motion for Summary Judgment is GRANTED.

II.

Defendant THC ORANGE COUNTY, LLC dba KINDRED HOSPITAL WESTMINSTER's Motion for Summary Judgment, or in the alternative, Summary Adjudication is DENIED.

Moving Party to give Notice.

Defendant Kindred Healthcare Operating, LLC (KHO)

moves for summary judgment pursuant to CCP § 437c, or in the alternative

summary adjudication as to Plaintiff LESTER BILES, by and through his purported

Successor-In-Interest, Teresa Sokol and TERESA SOKOL'S (Plaintiff) causes of action for: (1) Elder Abuse; (2) Willful Misconduct; and (3)

Wrongful Death on the grounds that Plaintiff Teresa Sokol does not have standing to sue and each cause of action lacks merit.

Defendant THC ORANGE COUNTY, LLC dba KINDRED

HOSPITAL WESTMINSTER (Kindred Westminster) moves for summary judgment pursuant to CCP § 437c, or in the alternative summary adjudication as to Plaintiff

LESTER BILES, by and through his purported Successor-In-Interest, Teresa Sokol and TERESA SOKOL'S (Plaintiff) causes of action for: (1)

Elder Abuse; (2) Willful Misconduct; and (3) Wrongful Death on the grounds that Plaintiff Teresa Sokol does not have standing to sue and each cause of action lacks merit.

BACKGROUND

This personal injury action was filed by

Plaintiff TERESA SOKOL, individually and as the surviving heir, personal representative, and/or successor-in-interest of Decedent, LESTER DALE BILES, and/or as administrator or executor of the Estate of Lester Dale Biles

("Plaintiff") on September 28, 2022. On October 5, 2023, the operative First

Amended Complaint ("FAC") was filed. Plaintiff alleges "LESTER DALE BILES

('DECEDENT') died on May 16, 2021, from injuries sustained

while a patient at KINDRED HOSPITAL WESTMINSTER (Kindred Westminster).... [¶]

DECEDENT also sustained injuries while a patient at MESA VERDE POST ACUTE...."

(FAC ¶¶1 and 4.) The FAC asserts the following causes of action:

(1) Negligence;

(2) Elder Abuse;

(3) Willful Misconduct; and

(4) Wrongful Death.

OBJECTIONS

Plaintiff's

objections to the declaration of Scott Graeser are OVERRULED.

REQUEST

FOR JUDICIAL NOTICE

Defendant

KHO's Request for Judicial Notice is GRANTED.

LEGAL

STANDARD

A party seeking summary judgment has the burden of producing evidentiary facts sufficient to entitle them to judgment as a matter of law. (CCP § 437c(c).) The moving party must make an affirmative showing that they are entitled to judgment irrespective of whether or not the opposing party files an opposition. (*Villa v. McFerren* (1995) 35 Cal.App.4th 733, 742743.) Thus, "the initial burden is always on the moving party to make a prima facie showing that there are no triable issues of material fact." (*Scalf v. D. B. Log Homes, Inc.* (2005) 128 Cal.App.4th 1510, 1519 (citing *Aguilar v. Atlantic Richfield Co.* (2001) 25 Cal.4th 826, 850.)

When a plaintiff seeks summary judgment, they must produce admissible evidence on each element of each cause of action on which judgment is sought. (CCP § 437c(p)(1).) When a defendant seeks summary judgment, they have the "burden of showing that a cause of action has no merit if the party has shown that one or more elements of the cause of action, even if not separately pleaded, cannot be established, or that there is a complete defense to the cause of action." (CCP § 437c(p)(2).)

The opposing party on a motion for summary judgment is under no evidentiary burden to produce rebuttal evidence until the moving party meets their initial movant's burden. (*Binder v. Aetna Life Insurance Company* (1999) 75 Cal.App.4th 832, 840.) Once the initial movant's burden is met, then the burden shifts to the opposing party to show, with admissible evidence, that there is a triable issue requiring the weighing procedures of trial. (CCP § 437c(p).) The opposing party may not simply rely on their allegations to show a triable issue but must present evidentiary facts that are substantial in nature and rise beyond mere speculation. (*Sangster v. Paetkau* (1998)

68Cal.App.4th 151, 162.) Summary judgment must be granted “if all the evidence submitted, and ‘all inferences reasonably deducible from the evidence’ and uncontradicted by other inferences or evidence, show that there is no triable issue as to any material fact and that the moving party is entitled to judgment as a matter of law.” (Adler v. Manor Healthcare Corp. (1992) 7Cal.App.4th1110, 1119.)

DISCUSSION

I.

Standing as to KHO and
Kindred Wesminster

California

Code of Civil Procedure section 367 provides that every action must be prosecuted in the name of the real party in interest, except as otherwise provided by statute. This requirement is not merely technical; it goes to the existence of a cause of action and the right to relief. Lack of standing, which concerns whether the plaintiff is the real party in interest, is properly raised by general demurrer and may be raised at any point in the proceedings.

Defendants KHO and Kindred Westminster argue that Plaintiff responded to Kindred Westminster's Demand for Production No. 6, stating that she is not in possession of any documents constituting Decedent's last will and testament. (SSUF 3.) Also as part of discovery, Kindred Westminster took the deposition of Plaintiff Theresa Sokol, wherein she testified as to the existence of Rhett Miles, Decedent's son, who was not a member of the action at that moment in time. (SSUF 4.) Subsequently, Plaintiffs filed a DOE Amendment to the operative Complaint in which they designated RHETT HARPER BILES (hereinafter referred to as “Rhett Biles”) from DOE 3 to a named nominal defendant. (SSUF 5.) During the subsequent videoconference discussion regarding said issue, Plaintiffs confirmed the lack of a valid will in addition to no appointment through probate, thus indicating an intent to obtain an assignment or waiver of rights from Rhett Biles to support Plaintiff Theresa Soko's status as the purported Successor in Interest and Heir. (SSUF 6.) However, no documentation reflecting a legally valid waiver, assignment of rights, probate code disclaimer, or stipulation has been produced or proposed. (SSUF 7.)

However, Plaintiff has testified “Q. Okay. And

as far as your understanding, do you know if you are your brother's successor in interest? A. There's no one else. There's – most of our family is passed. Q. Okay. A. And I've always taken care of my brother." (Exhibit "6," Depo. of Plaintiff, Pg. 17:15-21.) Plaintiff is the Decedent's older sister who resides in California. (Exhibit "6," Pg. 13:10-14; Pg. 14:1-8; Pg. 17:2-5.) At Kindred, she was the Decedent's power of attorney for medical decisions: "Q. Okay. During this time while he was at Kindred, were you, like, the medical decision-maker, the responsible party for him? A. I was his power of attorney for medical. Q. Okay. And is that pursuant to a Durable Power of Attorney that – A. Yes. Q. You had signed? Okay. ..." (Exhibit "6," Pg. 48:7-14.) Teresa Sokol is the only family contact identified in the Decedent's medical records. Teresa Sokol paid out-of-pocket for the Decedent's cremation and burial, in addition to requesting assistance from FEMA. (Exhibit "6," Pg. 63:20-25; Pg. 64; Pg. 65:1-21.)

Moreover, Rhett Biles declares he does not wish to either personally commence or maintain a survival action on the Decedent's behalf, nor actively participate in the prosecution of the wrongful death claims in this case. (Biles Decl., ¶¶ 3, 7.) He has therefore been named as a nominal Defendant.

Therefore, the Court finds that Plaintiff has standing to sue. Rhett Biles has consented to his aunt, Teresa Sokol, acting as Decedent's successor in interest for purposes of prosecuting, maintaining, and settling any survival causes of action on behalf of the Decedent, including but not limited to, claims under the Elder Abuse and Dependent Act. (Biles Decl., ¶ 4.) Moreover, the case docket demonstrates that on February 10, 2026, Biles was added as a nominal defendant.

II.

KHO's Motion for Summary Judgment

A.

KHO's Liability

In a parent-subsidiary relationship, the agency doctrine may bind a parent to the contracts of its subsidiary where, in addition to owning the subsidiary, the parent company exercises "sufficient control over the [subsidiary's] activities" such that the subsidiary becomes a "mere agen[t] or 'instrumentality' of the parent." (Cohen v. TNP 2008

KHO argues that KHO is a separate corporate entity from Kindred Westminster, that it does not own or operate Kindred Westminster, and that its relationship with Kindred Westminster is governed by an administrative and support services agreement that expressly preserves the entities' separate corporate identities and designates KHO as an independent contractor.

KHO provides the following evidence in support of its argument that KHO did not own, operate or provide clinical services or employees to Kindred Westminster during the period of Decedent's hospitalization (SSUF 12, 13, 14, 16, 18-20, 22.):

.
At all times pertinent to the allegations in Plaintiff's Complaint, there existed between KHO and Kindred Westminster a document entitled "Administrative and Support Services Agreement." Declaration of Graeser at Page 2, Lines 13-16 referencing "Administrative and Support Services Agreement" Exhibit 8.

.
The Administrative and Support Services Agreement between KHO and Kindred Westminster outlines the full extent of the business relationship between these two entities. Declaration of Graeser at Page 2, Lines 22-25 referencing "Administrative and Support Services Agreement" Exhibit 8.

.
Under this Agreement, KHO provided Kindred Westminster assistance with those administrative and support services enumerated in section 1.1 of the Administrative and Support Services Agreement. Graeser Declaration at Page 2, Lines 26- 28; and at Page 3, Lines 1-3, referencing Section 1.1 of the "Administrative and Support Services Agreement," Exhibit 8.

.
At no time pertinent to the allegations contained in the Complaint did KHO hold the operating license to Kindred Westminster. Graeser Declaration at Page 3, Lines 17- 18; Operating License for Kindred Westminster, KHO's Request for Judicial Notice, Exhibit 9.

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At no time pertinent to the allegations contained within the Complaint did KHO employees provide any healthcare services or treatment to any patient at Kindred Westminster, including Decedent. Graeser Declaration at Page 3, Lines 21- 22.

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KHO and Kindred Westminster are separate legal entities. Graeser Declaration at Page 3, Line 26.

The Court finds that KHO has met its movant's burden. KHO states that the only administrative and support services KHO provided to Kindred Westminster, its subsidiary, are outlined in Section 1.1 of the "Administrative and Support Services Agreement," that evidence states:

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"Effective as of January 1, 2006 (the "Effective Date"), Services Recipient hereby contracts with KHO to provide certain services, including information systems, clinical support, human resources management assistance, employee benefit plan development, training, reimbursement support, licensure and certification maintenance and support, finance, accounting, legal services and advice, insurance program management and advice, risk management services, compliance monitoring and support, payroll processing, accounts payable and receivable services, purchasing and facilities management support (collectively, the "Administrative and Support Services"), to Services Recipient to assist Services Recipient in the Operations, and KHO hereby accepts such contract, subject to the terms and provisions of this Agreement." (Exhibit 8.)

Moreover, Section 1.2 of the same agreement demonstrates that the parties explicitly intended that the subsidiary would not be an agent of KHO. Section 1.2 states:

.

"Services Recipient and KHO are, and intend act and perform as, independent contractors, and this Agreement does not create and is not intended to create any partnership, joint venture, management, agency or employment relationship between the parties. Services Recipient shall at all times and with respect to all elements

of the Operations retain and exercise sole and complete professional, administrative, business, pricing and operational responsibility and authority for and with respect to the Operations and shall discharge such responsibility and authority. KHO shall not provide any management or supervisory services to Services Recipient. It is expressly understood and agreed that each party shall be responsible for its own employees and that neither party shall make any claim against the other party for contribution or payment of work pay, vacation pay, sick leave, retirement benefits, social security contributions, disability or unemployment insurance benefits or employee benefits of any kind or nature with respect to its own employees. Each party shall be solely responsible for and shall comply, in all material respects, with all state and federal laws applicable to the performance of its responsibilities and the exercise of its authority hereunder; provided, however, that Services Recipient shall perform those obligations and responsibilities that must be performed by the party licensed to conduct the Operations pursuant to applicable state and federal laws and any resident or patient agreements.” (Exhibit 8.)

Therefore, the Court finds KHO has made a showing the relationship between KHO and Kindred Westminster was not that of an agent whereby KHO is liable for any of the alleged acts of Kindred Westminster as they related to Decedent. The Court finds that KHO has met its prima facie burden demonstrating it did not exercise “sufficient control over the [subsidiary’s] activities” such that the subsidiary was a “mere agen[t] or ‘instrumentality’ of the parent.” (Cohen v. TNP 2008 Participating Notes Program, LLC (2019) 31 Cal.App.5th 840, 862.)

Thus, the burden shifts to Plaintiff to raise a triable issue of material fact. Plaintiff argues that the Declaration of Scott Graeser (V.P. of Finance and Treasurer of ScionHealth, formerly known as Kindred Healthcare) offered as evidence to disprove liability is objectionable and should be stricken or disregarded because of its conclusory statements. (See Evidentiary Objections to Declaration of Scott Graeser filed concurrently herewith.) Moreover, Plaintiff argues that there are triable issues of fact as to KHO’s vicarious liability based on the agency relationship between KHO and THC. Plaintiff contends that KHO owns 100% of THC. (See Evidentiary Objections to Declaration of Scott Graeser filed concurrently herewith. Exhibit “9” is a copy of the license issued to THC – Orange County, LLC. KHO owns 100% of THC. (See www.cdph.ca.gov, California Health Facilities Information Database, Ownership/Operator Information.) Plaintiff argues that the existence of an agency is a factual question within the

province of the trier of fact. In addition, Graeser has failed to address a number of attributes of the parent-subsidary relationship which are material to the analysis of agency, according to the authority cited in the moving papers. "For example, do KHO and THC have common directors or officers? Does KHO guarantee THC's debts? Did KHO secure insurance coverage for THC? Is there a guarantee by KHO of THC's financial obligations? Has there been consolidation of financial reporting?" (Opp., pp. 16: 20-24.)

The Court finds that Plaintiff fails to raise a triable issue of material fact. As to Plaintiff's objection to the Graeser declaration, the Court finds Plaintiff's argument that Graeser's competency to testify to the matters stated has not been established unavailing. Plaintiff argues that given that Graeser works for ScionHealth, which appears to operate independently from Kindred Healthcare, it is unclear if Graeser worked for Kindred Healthcare prior to the transaction in 2021 where LifePoint Health and Kindred Healthcare established a new healthcare company operating under the name ScionHealth (per the www.scionhealth.com website in the news-stories section). However, Graeser's declaration, and the supporting evidence of the "Administrative and Support Services Agreement" sufficiently demonstrate Graeser's competency. Graeser's declaration states:

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I am the Vice President of Finance and Treasurer at ScionHealth, formerly known as Kindred Healthcare. As part of my job responsibilities, I have knowledge of KINDRED HEALTHCARE OPERATING, LLC'S (hereinafter referred to as "KHO") operations. I am responsible for understanding the corporate structure of KHO and its subsidiaries. (Graeser Decl., ¶ 1.)

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At all times KHO was incorporated in the State of Delaware, with its principal place of business in the State of Kentucky. KHO maintains sole membership interest of THC ORANGE COUNTY, LLC dba KINDRED HOSPITAL WESTMINSTER (hereinafter referred to as "Kindred Westminster"). (Graeser Decl., ¶ 3.)

.

At all times pertinent to the allegations in Plaintiff's Complaint, there existed between KHO and Kindred Westminster a document entitled "Administrative and Support Services Agreement." A true and

correct copy of the Administrative and Support Services Agreement executed between the parties is attached to KHO's Compendium of Exhibits as Exhibit 8. (Graeser Decl., ¶ 4.)

The Administrative and Support Services

Agreement between KHO and Kindred Westminster outlines the full extent of the business relationship between these two entities. Significantly, KHO neither owed, nor owes, any other duty to Kindred Westminster other than what is articulated in the Administrative and Support Services Agreement. (Graeser Decl., ¶ 5.)

Graeser's declaration establishes why and how he is competent to testify about the corporate structure of KHO and its subsidiaries. Additionally, the declaration attaches material evidence as to the relationship between the relevant entities.

Moreover, Plaintiff fails to provide evidence to rebut to the "Administrative and Support Services Agreement," which explicitly provides "Services Recipient and KHO are, and intend act and perform as, independent contractors, and this Agreement does not create and is not intended to create any partnership, joint venture, management, agency or employment relationship between the parties." (Exhibit 8.) It is immaterial that KHO owned Kindred Westminster when the Service Agreement provides that the latter was not an agent of the former. For the same reasons stated above, Plaintiff's arguments regarding joint venture and alter ego liability fail. Particularly, under the Administrative and Support Services Agreement, THC Orange County, LLC remained the sole legal entity exercising management and operational control over the day-to-day operations of Kindred Westminster. (SSUF 16, 19.) Consequently, KHO did not itself operate, manage or participate in the day-to-day operations of Kindred Westminster during the period of Decedent's hospitalization. (SSUF 16, 19.) KHO did not hold the operating license for Kindred Westminster or employ any individual at that hospital. (SSUF 18, 19.) At no time, pertinent to the allegations contained within Plaintiff's FAC, did KHO employees provide healthcare services or treatment to any patient at Kindred Westminster, including Decedent. (SSUF 20.) Additionally, KHO maintains separate and distinct corporate records from THC Orange County, LLC and is a separate legal entity. (SSUF 22.)

Accordingly, the motion for summary adjudication

as to KHO's liability is GRANTED.

B. 1st-3rd

CAUSE OF ACTION

FIRST CAUSE OF ACTION: The Elder Abuse Act

defines abuse as "[p]hysical abuse, neglect, financial abuse, abandonment, isolation, abduction, or other treatment with resulting physical harm or pain or mental suffering" (Welf. & Inst.Code, § 15610.07, subd. (a), italics added); or "[t]he deprivation by a care custodian of goods or services that are necessary to avoid physical harm or mental suffering" (id., § 15610.07, subd. (b)). The Act defines neglect as "[t]he negligent failure of any person having the care or custody of an elder or a dependent adult to exercise that degree of care that a reasonable person in a like position would exercise." (Id., § 15610.57, subd. (a)(1).)

SECOND CAUSE OF ACTION: In order to establish

willful misconduct, plaintiff must prove not only the elements of a negligence cause of action, that is, duty, breach of duty, causation, and damage, but also: (1) actual or constructive knowledge of the peril to be apprehended; (2) actual or constructive knowledge that injury is probable, as opposed to a possible, result of the danger; and (3) conscious failure to act to avoid the peril. (Doe v. United States Youth Soccer Assn., Inc. (2017) 8 Cal.App.5th 1118.)

THIRD CAUSE OF ACTION: The elements of the

cause of action for wrongful death are the tort, i.e., negligence or other wrongful act, the resulting death, and the damages, consisting of the pecuniary loss suffered by the heirs. (Code Civ. Proc., § 377.60 et seq.)

Each cause of action requires KHO to have

owed Decedent some duty of care. As stated above, there is no evidence supporting the contention that a custodial relationship existed between KHO and Decedent. Therefore, Plaintiff's causes of action fail as to KHO.

Accordingly, KHO's Motion for Summary Judgment as to the first through third causes is GRANTED.

III.

Kindred Westminster's Motion for Summary

Judgment

A. Elder Abuse

FIRST CAUSE OF ACTION: The Elder Abuse Act

defines abuse as “[p]hysical abuse, neglect, financial abuse, abandonment, isolation, abduction, or other treatment with resulting physical harm or pain or mental suffering” (Welf. & Inst.Code, § 15610.07, subd. (a), italics added); or “[t]he deprivation by a care custodian of goods or services that are necessary to avoid physical harm or mental suffering” (id., § 15610.07, subd. (b)). The Act defines neglect as “[t]he negligent failure of any person having the care or custody of an elder or a dependent adult to exercise that degree of care that a reasonable person in a like position would exercise.” (Id., § 15610.57, subd. (a)(1).)

[S]everal

factors [] must be present for conduct to constitute neglect within the meaning of the Elder Abuse Act and thereby trigger the enhanced remedies available under the Act. The plaintiff must allege (and ultimately prove by clear and convincing evidence) facts establishing that the defendant: (1) had responsibility for meeting the basic needs of the elder or dependent adult, such as nutrition, hydration, hygiene or medical care; (2) knew of conditions that made the elder or dependent adult unable to provide for his or her own basic needs; and (3) denied or withheld goods or services necessary to meet the elder or dependent adult’s basic needs, either with knowledge that injury was substantially certain to befall the elder or dependent adult (if the plaintiff alleges oppression, fraud or malice) or with conscious disregard of the high probability of such injury (if the plaintiff alleges recklessness). The plaintiff must also allege (and ultimately prove by clear and convincing evidence) that the neglect caused the elder or dependent adult to suffer physical harm, pain or mental suffering. Finally, the facts constituting the neglect and establishing the causal link between the neglect and the injury “must be pleaded with particularity,” in accordance with the pleading rules governing statutory claims. (Carter v. Prime Healthcare Paradise Valley LLC (2011) 198 Cal.App.4th 396, 406-407, citations omitted.)

Acts

which are prohibited under the Elder Abuse Act provisions of the Welfare and Institute Code “do not include acts of simple professional negligence, but

refer to forms of abuse or neglect performed with some state of culpability greater than mere negligence.” (Covenant Care, Inc. v. Superior Court (2004) 32 Cal.4th 771, 781 (Covenant Care).) To constitute abuse and neglect within the meaning of the Elder Abuse Act, thereby triggering the enhanced remedies available under the Act, a plaintiff “must demonstrate by clear and convincing evidence that defendant is guilty of something more than negligence; he or she must show reckless, oppressive, fraudulent, or malicious conduct,” with recklessness referring “to a subjective state of culpability greater than simple negligence, which has been described as a ‘deliberate disregard’ of the ‘high degree of probability’ that an injury will occur,” and ‘[o]ppression, fraud and malice [involving] intentional or conscious wrongdoing of a despicable or injurious nature.’” (Sababin v. Superior Court (2006) 144 Cal.App.4th 81, 88-89, quotation marks omitted.)

“[N]eglect refers not to the substandard performance of medical services but, rather, to the failure of those responsible for attending to the basic needs and comforts of elderly or dependent adults, regardless of their professional standing, to carry out their custodial obligations.’ Thus, the statutory definition of ‘neglect’ speaks not of the undertaking of medical services, but of the failure to provide medical care.” (Covenant Care, Inc. v. Superior Court (2004) 32 Cal.4th 771, 783.)

Kindred Westminster argues that Plaintiff’s claims against Kindred Westminster relate to medical, not custodial care. Moreover, Kindred Westminster argues that even assuming that the care at issue could be characterized as custodial, Plaintiffs still cannot establish an elder abuse claim because there is no evidence that Kindred Westminster withheld or denied basic necessary services. Upon admission, Kindred Westminster assessed Decedent’s skin integrity, identified multiple wounds that were present on admission and/or community-acquired, and implemented a comprehensive wound care plan. Decedent was maintained on pressure redistribution surfaces, repositioned and offloaded, received ongoing wound treatment, underwent serial wound assessments, and was regularly monitored for skin breakdown. Plaintiffs’ allegations regarding repositioning and wound management challenge the adequacy of care provided, not the absence of care. Therefore, Plaintiffs’ evidence reflects a disagreement regarding the adequacy of care provided to a medically complex patient suffering from septic shock, aspiration pneumonia, osteomyelitis, profound immobility, and numerous severe wounds present upon

admission. Such allegations may implicate professional negligence, but they do not constitute the withholding or denial of basic custodial services required to support an elder abuse claim.

The Court finds that Kindred Westminster has met its prima facie burden given the evidence demonstrates Kindred Westminster assessed Decedent upon admission, identified multiple preexisting and community acquired wounds, implemented physician-directed wound care interventions, maintained pressure redistribution surfaces, provided repositioning and offloading, monitored nutrition and hydration, administered antibiotics, obtained specialty consultations, transferred Decedent to the ICU when his condition deteriorated, and provided resuscitative efforts during his final decline.

The burden shifts to Plaintiff to demonstrate a triable issue of material fact. Plaintiff argues that Kindred Westminster's staff performed custodial functions and provided professional medical care to the Decedent. (*Covenant Care, Inc. v. Superior Court* (2004) 32 Cal.4th 771, 786.) Kindred Westminster assumed the responsibility for attending to the Decedent's basic needs (including but not limited to personal hygiene, nutrition, hydration, bathing/showering, dressing, oral care, etc.), while he was a patient at the hospital dating from April 29, 2021, through May 16, 2021. Plaintiff submits evidence of the following:

.

The Defendant denied or withheld goods or services necessary to meet the Decedent's basic needs. (Dr. Bain Decl., ¶¶ 32-67 re: deficient pressure-relief care and failure to timely escalate care.)

.

Dr. Bain has opined that Kindred Westminster breached the applicable standard of care and this constituted custodial neglect. (Dr. Bain Decl., ¶¶ 32, 92-95.)

The Court determines that Plaintiff has raised a triable issue of material fact as to elder abuse. Plaintiff sufficiently evidences that Kindred Westminster had the responsibility to meet the basic needs of Decedent, including nutrition, hydration, hygiene, and medical care because at the time of arrival at Kindred, he was critically ill after septic shock, pneumonia, and multiple severe pressure wounds stage 4 left hip pressure injury, multiple deep tissue pressure injuries (DTPI) and

unstageable wounds (hips, buttock, heels, feet, thigh), with a high Braden risk score of 9. (Dr. Bain Decl., ¶ 17.)

Indeed, Decedent was completely dependent on Kindred Westminster given Decedent was bedbound with contractures, unable to participate, and at high aspiration risk with severe dysphagia. Speech pathology also documented severe dysphagia. Tube feeding was continued, but with recurrent aspiration events. Chest imaging showed new/worsening pneumonia. He had multiple ICU transfers following aspiration episodes. (Dr. Bain Decl., ¶¶ 20-21.) Moreover, Plaintiff evidences that Kindred Westminster failed to meet the applicable standard of custodial care under the Elder Abuse Act. Dr. Bain opines:

.

Kindred

Westminster breached the standard of care, to a reasonable degree of medical probability, by the following:

o

failing

to implement a strict q2h turning and repositioning plan for a bedbound, contractured, severely debilitated patient with multiple pressure injuries and a Braden score of 9;

o

failing

to reliably carry out even Kindred's own less-protective q2-4 hour repositioning plan, with the record reflecting approximately 183 turn opportunities, 115 performed turns, and 68 missed turns, or only approximately 62.84% compliance;

o

failing

to adequately offload existing wounds, including repeated back or supine positioning despite bilateral hip and buttock pressure injuries and prior Hoag directives to avoid positioning on pressure injuries; • treating the specialty bed or support surface as though it could substitute for active repositioning, when specialty surfaces supplement but do not replace turning, offloading, and skin monitoring;

o

failing

to adequately respond to a persistent severe stage 4 left hip wound with undermining and known concern for osteomyelitis;

o

failing

to evaluate the persistent stage 4 wound and left femoral osteomyelitis as a potential source of sepsis when Mr. Biles deteriorated, despite the fact that the infectious workup documented blood, urine, and sputum cultures but no wound cultures during the Kindred admission;

o

failing

to safely manage severe dysphagia, recurrent aspiration, feeding intolerance, high residuals, and eventual TPN dependence as signs of medical instability exceeding routine LTACH wound-care management;

o

failing

to timely escalate care or document meaningful consideration of transfer back to an acute-care hospital despite repeated signs of acute-care-level instability, including lactic acidosis, hypotension, GI bleeding, aspiration events, ICU transfers, vasopressor dependence, respiratory failure, and code blue deterioration; and,

o

failing

to adequately communicate the seriousness of Mr. Biles' decline to family, as the Kindred records do not document meaningful family meetings or family notification despite repeated serious deterioration and ICU-level events.

(Dr.

Bain Decl., ¶ 93.)

Moreover,

Dr. Bain opines that Kindred Westminster's omissions caused or contributed to additional pressure-related tissue injury, persistence of severe wound burden, impaired healing, increased pain and suffering, increased infection risk, and worsening physiologic decline. (See Dr. Bain Decl., ¶¶ 82-85.)

On

this basis, the Court finds that Plaintiff has presented sufficient evidence to raise a triable issue of material fact as to the elder abuse cause of action.

Accordingly,
summary judgment is DENIED. Summary
adjudication as to the elder abuse cause of action is DENIED.

B.

Willful

Misconduct

In order to establish willful misconduct, plaintiff must prove not only the elements of a negligence cause of action, that is, duty, breach of duty, causation, and damage, but also: (1) actual or constructive knowledge of the peril to be apprehended; (2) actual or constructive knowledge that injury is probable, as opposed to a possible, result of the danger; and (3) conscious failure to act to avoid the peril. (*Doe v. United States Youth Soccer Assn., Inc.* (2017) 8 Cal.App.5th 1118.) '[W]illful misconduct is not marked by a mere absence of care. Rather, it "involves a more positive intent actually to harm another or to do an act with a positive, active and absolute disregard of its consequences. (Id. at 1140.)

Kindred

Westminster argues that there is no evidence that Kindred Westminster engaged in intentional misconduct, withheld necessary care, or acted with conscious disregard for Decedent's condition. The undisputed record shows that Decedent was admitted from Hoag Memorial Hospital with septic shock, aspiration pneumonia, urinary tract infection, femoral osteomyelitis, dysphagia requiring PEG feeding, and numerous serious wounds already present and/or documented as community acquired, along with significant comorbidities impairing healing and increasing risk of decline. The evidence further establishes that Kindred Westminster actively treated Decedent throughout his admission, including performing a skin assessment upon admission, implementing a comprehensive wound care plan, maintaining pressure redistribution surfaces, repositioning and offloading, providing ordered wound care, managing incontinence, addressing nutritional needs, administering antibiotics, and obtaining multiple specialty consultations. The records also reflect ICU management during deterioration, adjustments to tube feeding due to aspiration concerns, and resuscitative

efforts when Decedent acutely decompensated. There is likewise no evidence that Decedent's wounds worsened or that new pressure injuries developed. Dr. Klein opines to a reasonable degree of medical probability that the wounds remained stable or improved, no new injuries developed, and the care met the standard of care— opinions that negate any claim of willful or reckless conduct.

The Court finds that Kindred Westminster has met its prima facie burden. The burden shifts to Plaintiff.

In opposition, Plaintiff argues that Kindred Westminster knew of Decedent's medical conditions (see Exhibit "1," History and Physical dated April 29, 2021), knew that the immobile and bedbound Decedent had numerous ulcers and that the Decedent was at risk of pressure ulcers (see Exhibit "2," bates stamp 458, showing a Braden scale of 9 and "at high risk"), yet failed to implement pressure ulcer preventive measures (i.e., two-hourly turning and repositioning, proper offloading principles, etc.) during his hospitalization which resulted in additional pressure-related tissue injury, persistence of severe wound burden, impaired healing, increased pain and suffering, increased infection risk, and worsening physiologic decline, as discussed in Dr. Bain's declaration. According to Plaintiff the facts show that the Defendant was aware of the risks of Decedent's condition (Decedent was an "extraordinarily high-risk patient," (Dr. Bain, ¶ 33), so the Defendant knew or should have known that it was probable, instead of merely possible, that Decedent would be harmed. Moreover, Dr. Bain opines that Kindred Westminster's omissions caused or contributed to additional pressure-related tissue injury, persistence of severe wound burden, impaired healing, increased pain and suffering, increased infection risk, and worsening physiologic decline. (See Dr. Bain Decl., ¶¶ 82-85.)

The Court finds that Plaintiff has raised a triable issue of material fact. As stated above, a question of fact remains whether Kindred Westminster's treatment of Decedent's wounds amounted to a conscious failure to act to avoid the peril against Decedent. (*Doe v. United States Youth Soccer Assn., Inc.* (2017) 8 Cal.App.5th 1118.).

Accordingly, summary adjudication as to the willful misconduct cause of action is DENIED.

C. Wrongful Death

To prevail on a claim

of wrongful death, a plaintiff must prove (1) a wrongful act or neglect on the part of one or more persons, that is, negligence, that (2) causes (3) the death of another person. (Musgrove v. Silver (2022) 82 Cal.App.5th 694, as modified on denial of reh'g (Sept. 13, 2022).)

As stated above, Plaintiff has adequately

raised a triable issue of material fact as to professional and/or medical negligence therefore this cause of action survives. (Dr. Bain Decl., ¶ 93.)

Accordingly,

summary adjudication as to the wrongful death cause of action is DENIED.